CLIENT CODE : C000021944

CLIENT'S NAME AND ADDRESS :
S.S. WELLNESS CENTRE - JODHPUR
GROUND FLOOR,
C-22,SHASTRI NAGAR NEAR CENTRAL ACADEMY SCHOOL
JODHPUR 342001
RAJASTHAN INDIA
0291-2646000 291-2644000

Cert. No. MC-2015

SRL LIMITED
SRL,REFERENCE LAB, GP-26, MARUTI INDUSTRIAL ESTATE,UDYOG VIHAR,SECTOR-18,
GURGAON, 122015
HARYANA, INDIA
Tel : 1800-222-000, 1800-102-8282, Fax : 0124
U74899PB1995PLC045956
Email : connect@srl.in

PATIENT NAME : LAKHAN

PATIENT ID :

ACCESSION NO : **0009RE038594** AGE : 15 Years SEX : Male DATE OF BIRTH :

DRAWN : 18-05-2018 00:00 RECEIVED : 19-05-2018 09:31 REPORTED : 19-05-2018 13:57

REFERRING DOCTOR : DR. PURSHOTAM DAN

CLIENT PATIENT ID :

Test Report Status **Final**

	Results		Biological Reference Interval	Units
COMPLETE CARE TOTAL				
COMPLETE BLOOD COUNT				
RED BLOOD CELL COUNT METHOD : COULTER IMPEDENCE PRINCIPLE	4.98		4.5 - 5.5	mil/µL
HEMOGLOBIN METHOD : PHOTOMETRIC MEASUREMENT	14.2		13.0 - 17.0	g/dL
HEMATOCRIT METHOD : CALCULATED PARAMETER	42.3		40 - 50	%
MEAN CORPUSCULAR VOL METHOD : DERIVED PARAMETER	84.8		83.0 - 101.0	fL
MEAN CORPUSCULAR HGB. METHOD : CALCULATED PARAMETER	28.5		27.0 - 32.0	pg
MEAN CORPUSCULAR HEMOGLOBIN CONCENTRATION METHOD : CALCULATED PARAMETER	33.6		31.5 - 34.5	
RED CELL DISTRIBUTION WIDTH METHOD : DERIVED PARAMETER	13.9		11.6 - 14.0	%
PLATELET COUNT METHOD : IMPEDENCE / PLATELET HISTOGRAM	**125**	**Low**	150 - 410	thou/µL
MEAN PLATELET VOLUME METHOD : DERIVED PARAMETER	**13.1**	**High**	6.8 - 10.9	fL
WHITE BLOOD CELL COUNT METHOD : COULTER IMPEDENCE PRINCIPLE	5.00		4.0 - 10.0	thou/µL
WBC DIFFERENTIAL COUNT				
SEGMENTED NEUTROPHILS METHOD : VCS TECHNOLOGY	45		40 - 80	%
EOSINOPHILS METHOD : VCS TECHNOLOGY	02		1 - 6	
LYMPHOCYTES METHOD : VCS TECHNOLOGY	**50**	**High**	20 - 40	%
MONOCYTES METHOD : VCS TECHNOLOGY	03		2 - 10	%
BASOPHILS METHOD : VCS TECHNOLOGY	00		< 1 - 2	%
DIFFERENTIAL COUNT PERFORMED ON:	EDTA SMEAR			

PERIPHERAL SMEAR EXAM, EDTA WHOLE BLOOD

RBC

PREDOMINANTLY NORMOCYTIC NORMOCHROMIC PICTURE.

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REFERRING DOCTOR : DR. PURSHOTAM DAN REPORTED : 19-05-2018 13:57

Test Report Status **Final** CLIENT PATIENT ID :

Test	Results		Biological Reference Interval	Units
METHOD : MICROSCOPIC EXAMINATION				
WBC	MILD RELATIVE LYMPHOCYTOSIS NOTED.			
METHOD : MICROSCOPIC EXAMINATION				
PLATELETS	MILD THROMBOCYTOPENIA NOTED.			
TOTAL IRON BINDING CAPACITY, SERUM				
IRON	**203**	**High**	65 - 175	µg/dL
METHOD : SPECTROPHOTOMETRY				
TOTAL IRON BINDING CAPACITY	422		250 - 450	µg/dL
METHOD : SPECTROPHOTOMETRY				
% SATURATION	**48.1**	**High**	13 - 45	%
METHOD : SPECTROPHOTOMETRY				
GLUCOSE, FASTING, PLASMA				
GLUCOSE, FASTING, PLASMA	77		74 - 99	mg/dL
METHOD : SPECTROPHOTOMETRY				
GLYCOSYLATED HEMOGLOBIN, EDTA WHOLE BLOOD				
GLYCOSYLATED HEMOGLOBIN (HBA1C)	5.1		Non-diabetic: < 5.7 Pre-diabetics: 5.7 - 6.4 Diabetics: > or = 6.5 ADA Target: 7.0 Action suggested: > 8.0	%
MEAN PLASMA GLUCOSE	99.7		< 116.0	mg/dL
LIVER FUNCTION PROFILE, SERUM				
BILIRUBIN, TOTAL	0.8		0.3 - 1.2	mg/dL
METHOD : SPECTROPHOTOMETRY				
BILIRUBIN, DIRECT	0.10		0.0 - 0.2	mg/dL
METHOD : SPECTROPHOTOMETRY				
TOTAL PROTEIN	7.1		6.4 - 8.2	g/dL
METHOD : SPECTROPHOTOMETRY				
ALBUMIN	4.2		3.4 - 5.0	g/dL
METHOD : SPECTROPHOTOMETRY				
GLOBULIN	2.9		2.0 - 4.1	G/DL
METHOD : SPECTROPHOTOMETRY				
ALBUMIN/GLOBULIN RATIO	1.5		1.0 - 2.1	RATIO
METHOD : SPECTROPHOTOMETRY				
ASPARTATE AMINOTRANSFERASE (AST/SGOT)	30		15 - 37	U/L
METHOD : SPECTROPHOTOMETRY				
ALANINE AMINOTRANSFERASE (ALT/SGPT)	22		< 45.0	U/L
METHOD : SPECTROPHOTOMETRY				

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PATIENT NAME : LAKHAN

ACCESSION NO : **0009RE038594** AGE : 15 Years SEX : Male PATIENT ID :

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Test Report Status **Final** CLIENT PATIENT ID :

	Results		Biological Reference Interval	Units
ALKALINE PHOSPHATASE METHOD : SPECTROPHOTOMETRY	**120**	**Low**	130 - 525	U/L
GAMMA GLUTAMYL TRANSFERASE (GGT) METHOD : SPECTROPHOTOMETRY	21		15 - 85	U/L
LACTATE DEHYDROGENASE METHOD : SPECTROPHOTOMETRY	**237**	**High**	100 - 190	U/L
25 - HYDROXYVITAMIN D, SERUM				
25 - HYDROXYVITAMIN D METHOD : CHEMILUMINESCENCE	20.59		Deficiency: < 15.0 Insufficiency: 15.0 - < 20.0 Sufficiency: 20.0 -100.0 Toxicity > 100.0	ng/mL
CALCIUM, SERUM				
CALCIUM METHOD : SPECTROPHOTOMETRY	8.7		8.5 - 10.1	mg/dL
VITAMIN B12 LEVEL, SERUM				
VITAMIN B12 METHOD : CHEMILUMINESCENCE	**185.0**	**Low**	211.0 - 911.0	pg/mL
*** THYROID PANEL, SERUM**				
T3 METHOD : CHEMILUMINESCENCE	116.13		86.0 - 192.0	ng/dl
T4 METHOD : CHEMILUMINESCENCE	6.00		5.5 - 11.1	µg/dl
TSH 3RD GENERATION METHOD : CHEMILUMINESCENCE	2.070		0.51 - 4.94	µIU/mL
CORONARY RISK PROFILE (LIPID PROFILE), SERUM				
CHOLESTEROL METHOD : SPECTROPHOTOMETRY	137		< 200 Desirable 200 - 239 Borderline High >/= 240 High	mg/dL
TRIGLYCERIDES METHOD : SPECTROPHOTOMETRY	52		< 150 Normal 150 - 199 Borderline High 200 - 499 High >/=500 Very High	mg/dL
HDL CHOLESTEROL METHOD : SPECTROPHOTOMETRY	55		< 40 Low >/=60 High	mg/dL

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REFERRING DOCTOR : DR. PURSHOTAM DAN CLIENT PATIENT ID :

Test Report Status **Final**	**Results**		**Biological Reference Interval**	**Units**
DIRECT LDL CHOLESTEROL METHOD : SPECTROPHOTOMETRY	74.00		< 100 Optimal 100 - 129 Near or above optimal 130 - 159 Borderline High 160 - 189 High >/= 190 Very High	mg/dL
NON HDL CHOLESTEROL	82		Desirable: Less than 130 Above Desirable: 130 - 159 Borderline High: 160 - 189 High: 190 - 219 Very high: > or = 220	mg/dL
CHOL/HDL RATIO	**2.5**	**Low**	3.3 - 4.4 Low Risk 4.5 - 7.0 Average Risk 7.1 - 11.0 Moderate Risk > 11.0 High Risk	
LDL/HDL RATIO METHOD : SPECTROPHOTOMETRY	1.4		0.5 - 3.0 Desirable/Low Risk 3.1 - 6.0 Borderline/Moderate Risk >6.0 High Risk	
VERY LOW DENSITY LIPOPROTEIN METHOD : SPECTROPHOTOMETRY	10.4		</= 30.0	mg/dL
SERUM BLOOD UREA NITROGEN				
BLOOD UREA NITROGEN METHOD : SPECTROPHOTOMETRY	9.0		5 - 18	mg/dL
CREATININE, SERUM				
CREATININE METHOD : SPECTROPHOTOMETRY	0.60		0.50 - 1.00	mg/dL
BUN/CREAT RATIO				
BUN/CREAT RATIO METHOD : SPECTROPHOTOMETRY	15.00		5.00 - 15.00	
URIC ACID, SERUM				
URIC ACID METHOD : SPECTROPHOTOMETRY	3.9		3.5 - 7.2	mg/dL
TOTAL PROTEIN, SERUM				
TOTAL PROTEIN METHOD : SPECTROPHOTOMETRY	7.1		6.4 - 8.2	g/dL
ALBUMIN, SERUM				
ALBUMIN METHOD : SPECTROPHOTOMETRY	4.2		3.4 - 5.0	g/dL
GLOBULIN				
GLOBULIN	2.9		2.0 - 4.1	g/dL

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CLIENT PATIENT ID :

Test Report Status **Final**

	Results	Biological Reference Interval	Units
METHOD : SPECTROPHOTOMETRY			
ELECTROLYTES (NA/K/CL), SERUM			
SODIUM	141	136 - 145	mmol/L
POTASSIUM	4.9	3.50 - 5.10	mmol/L
CHLORIDE	102	98 - 107	mmol/L
URINALYSIS			
REMARKS	TEST CANCELLED AS SPECIMEN NOT RECEIVED		
MAGNESIUM, SERUM			
MAGNESIUM	2.2	1.7 - 2.3	mg/dL
METHOD : SPECTROPHOTOMETRY			

Interpretation(s)
COMPLETE BLOOD COUNT-
The cell morphology is well preserved for 24hrs. However after 24-48 hrs a progressive increase in MCV and HCT is observed leading to a decrease in MCHC. A direct smear is recommended for an accurate differential count and for examination of RBC morphology.
TOTAL IRON BINDING CAPACITY, SERUM-
Total iron binding capacity (TIBC) measures the blood's capacity to bind iron with transferrin and thus is an indirect way of assessing transferrin level.
Taken together with serum iron and percent transferrin saturation this test is performed when they is a concern about anemia, iron deficiency or iron deficiency anemia.
However, because the liver produces transferrin, alterations in liver function (such as cirrhosis, hepatitis, or liver failure) must be considered when performing this test.
Increased in:
- iron deficiency
- acute and chronic blood loss
- acute liver damage
- progesterone birth control pills

Decreased in:
- hemochromatosis
- cirrhosis of the liver
- thalassemia
- anemias of infection and chronic diseases
- nephrosis
- hyperthyroidism

The percent Transferrin saturation = Serum Iron/TIBC x 100
Unsaturated Binding Capacity (UIBC)=TIBC - Serum Iron.
Limitations: Estrogens and oral contraceptives increase TIBC and Asparaginase, chloramphenicol, corticotropin, cortisone and testosterone decrease the TIBC level.

Reference:
1.Tietz Textbook of Clinical Chemistry and Molecular Diagnostics, edited by Carl A Burtis, Edward R.Ashwood, David E Bruns, 4th Edition, Elsevier publication, 2006, 563, 1314-1315.
2. Wallach's Interpretation of Diagnostic tests, 9th Edition, Ed Mary A Williamson and L Michael Snyder. Pub Lippincott Williams and Wilkins, 2011, 234-235.
GLUCOSE, FASTING, PLASMA-
ADA 2012 guidelines for adults as follows:
Pre-diabetics: 100 - 125 mg/dL
Diabetic: > or = 126 mg/dL

(Ref: Tietz 4th Edition & ADA 2012 Guidelines)
GLYCOSYLATED HEMOGLOBIN, EDTA WHOLE BLOOD-
GLYCOSYLATED HEMOGLOBIN, EDTA WHOLE BLOOD
Glycosylated hemoglobin (GHb) has been firmly established as an index of long-term blood glucose concentrations and as a measure of the risk for the development of complications in patients with diabetes mellitus. Formation of GHb is essentially irreversible, and the concentration in the blood depends on both the life span of the red blood cell (average 120 days) and the blood glucose concentration. Because the rate of formation of GHb is directly proportional to the concentration of glucose in the blood, the GHb concentration represents the integrated values for glucose over the preceding 6-8 weeks.
Any condition that alters the life span of the red blood cells has the potential to alter the GHb level. Samples from patients with hemolytic anemias will exhibit decreased glycated hemoglobin values due to the shortened life span of the red cells. This effect will depend upon the severity of the anemia. Samples from patients with polycythemia or post-splenectomy may exhibit increased glycated hemoglobin values due to a somewhat longer life span of the red cells.
Glycosylated hemoglobins results from patients with HbSS, HbCC, and HbSC and HbD must be interpreted with caution, given the pathological processes, including anemia, increased red cell turnover, transfusion requirements, that adversely impact HbA1c as a marker of long-term glycemic control. In these conditions, alternative forms of testing such as glycated serum protein (fructosamine) should be considered.
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• Rapid weight loss.
Gout
Lesch nyhan syndrome.
Type 2 DM.
Metabolic syndrome.

Causes of decreased levels
• Low Zinc Intake
• OCP's
• Multiple Sclerosis

Nutritional tips to manage increased Uric acid levels
• Drink plenty of fluids
• Limit animal proteins
• High Fibre foods
• Vit C Intake
• Antioxidant rich foods

TOTAL PROTEIN, SERUM-
Serum total protein,also known as total protein, is a biochemical test for measuring the total amount of protein in serum..Protein in the plasma is made up of albumin and globulin

Higher-than-normal levels may be due to: Chronic inflammation or infection, including HIV and hepatitis B or C, Multiple myeloma, Waldenstrom's disease
Lower-than-normal levels may be due to: Agammaglobulinemia, Bleeding (hemorrhage),Burns,Glomerulonephritis, Liver disease, Malabsorption, Malnutrition, Nephrotic syndrome,Protein-losing enteropathy etc.

ALBUMIN, SERUM-
Human serum albumin is the most abundant protein in human blood plasma. It is produced in the liver. Albumin constitutes about half of the blood serum protein. Low blood albumin levels (hypoalbuminemia) can be caused by: Liver disease like cirrhosis of the liver, nephrotic syndrome, protein-losing enteropathy, Burns, hemodilution, increased vascular permeability or decreased lymphatic clearance,malnutrition and wasting etc.

ELECTROLYTES (NA/K/CL), SERUM-
Sodium levels are Increased in dehydration, cushing's syndrome, aldosteronism & decreased in Addison's disease, hypopituitarism,liver disease. Hypokalemia (low K) is common in vomiting, diarrhea, alcoholism, folic acid deficiency and primary aldosteronism. Hyperkalemia may be seen in end-stage renal failure, hemolysis, trauma, Addison's disease, metabolic acidosis, acute starvation, dehydration, and with rapid K infusion.Chloride is increased in dehydration, renal tubular acidosis (hyperchloremia metabolic acidosis), acute renal failure, metabolic acidosis associated with prolonged diarrhea and loss of sodium bicarbonate, diabetes insipidus, adrenocortical hyperfuction, salicylate intoxication and with excessive infusion of isotonic saline or extremely high dietary intake of salt.Chloride is decreased in overhydration, chronic respiratory acidosis, salt-losing nephritis, metabolic alkalosis, congestive heart failure, Addisonian crisis, certain types of metabolic acidosis, persistent gastric secretion and prolonged vomiting.

URINALYSIS-Routine urine analysis assists in screening and diagnosis of various metabolic, urological, kidney and liver disorders
Protein: Elevated proteins can be an early sign of kidney disease. Urinary protein excretion can also be temporarily elevated by strenuous exercise, orthostatic proteinuria, dehydration, urinary tract infections and acute illness with fever
Glucose: Uncontrolled diabetes mellitus can lead to presence of glucose in urine. Other causes include pregnancy, hormonal disturbances, liver disease and certain medications.
Ketones: Uncontrolled diabetes mellitus can lead to presence of ketones in urine. Ketones can also be seen in starvation, frequent vomiting, pregnancy and strenuous exercise.
Blood: Occult blood can occur in urine as intact erythrocytes or haemoglobin, which can occur in various urological, nephrological and bleeding disorders.
Leukocytes: An increase in leukocytes is an indication of inflammation in urinary tract or kidneys. Most common cause is bacterial urinary tract infection.
Nitrite: Many bacteria give positive results when their number is high. Nitrite concentration during infection increases with length of time the urine specimen is retained in bladder prior to collection.
pH: The kidneys play an important role in maintaining acid base balance of the body. Conditions of the body producing acidosis/ alkalosis or ingestion of certain type of food can affect the pH of urine.
Specific gravity: Specific gravity gives an indication of how concentrated the urine is. Increased specific gravity is seen in conditions like dehydration, glycosuria and proteinuria while decreased specific gravity is seen in excessive fluid intake, renal failure and diabetes insipidus.
Bilirubin: In certain liver diseases such as biliary obstruction or hepatitis, bilirubin gets excreted in urine.
Urobilinogen: Positive results are seen in liver diseases like hepatitis and cirrhosis and in cases of hemolytic anemia

MAGNESIUM, SERUM-
Moderate or severe magnesium deficiency is usually due to losses of magnesium from gastrointestinal tract or kidneys as in vomiting and diarrhoea in former and alcohol, diabetes mellitus (osmotic diuresis), loop diuretics (furosemide) and aminoglycoside antibiotics in latter.

Symptomatic hypermagnesemia is almost always caused by excessive intake with concomitant renal failure, thereby decreasing the ability of the kidneys to excrete excess magnesium.

Magnesium concentration in erythrocytes are approximately three times those of serum. Conversion factors for the units used to express magnesium concentration are:
mg/dl= meq/l x 1.22 = mmol/l x 2.43

****End Of Report****

Please visit www.srlworld.com for related Test Information for this accession
TEST MARKED WITH '*' ARE OUTSIDE THE NABL ACCREDITED SCOPE OF THE LABORATORY.